Bevacizumab + capecitabine

This combination treats metastatic or unresectable colorectal cancer when an irinotecan- or oxaliplatin-based combination is unsuitable. Its aims may include cancer control, symptom relief and longer survival.

Advanced colorectal cancer · NCCP 00623 v3a · Document revised 4 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

My overall treatment

Bevacizumab infusion on Day 1; capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Follow your individual prescription.

21 day view

Treatment timing in this window

Bevacizumab infusion — Day 1

Capecitabine tablets — Day 1–14

21-day tablet and infusion cycle
Bevacizumab infusion on Day 1; capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Follow your individual prescription.

Review
Course length, response scans and next steps follow your individual plan.

A typical 21-day cycle

Day 1–14	Day 15–21
Treatment days	Planned tablet-free days

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) and baseline bloods** Required by regimen	Confirm DPD testing, blood count, kidney and liver results before starting.
During	**Blood count / kidneys / liver** Required by regimen	Before each cycle. Review symptoms and medicines.
Before / during	**Blood pressure / urine protein** Required by regimen	Check at baseline and during treatment. Blood pressure before each cycle and after treatment; regular urine dipstick.
Before / if needed	**Heart assessment / echocardiogram (heart ultrasound) (ECHO)** Baseline review; ECHO if indicated	Heart history and examination before treatment. Consider ECHO after chest-wall radiation or earlier anthracycline treatment.
If warfarin	**international normalised ratio (blood-clotting test) (INR)** Conditional on anticoagulant	Weekly until warfarin dose stable, then before each cycle.
After / review	**Recovery, scans / markers** Team-agreed plan	Follow the individual cancer plan; this regimen does not prescribe a universal scan or marker interval.

Between treatments

- Report fever, sudden illness, diarrhoea, mouth pain or poor intake promptly.
- Symptoms can occur during treatment, between visits and after completion.

Confirm your prescription, supportive medicines, tests and working contact numbers.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Mouth

Capecitabine

Painful ulcers or reduced intake: stop tablets and call.

3 Bowel / stoma

Capecitabine

Significant diarrhoea, increased output or dehydration needs prompt advice.

5 Blood / infection

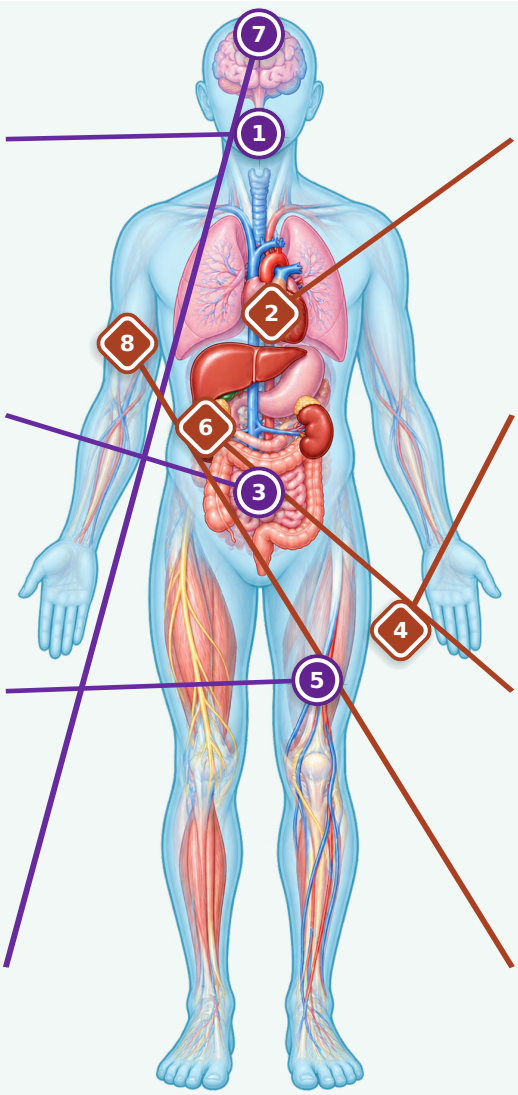
Capecitabine

Fever, shivers or sudden illness: contact oncology immediately.

7 Head / blood pressure

Bevacizumab

Severe headache, visual change or sudden weakness needs urgent help.



2 Chest

Capecitabine

New chest pain: stop tablets and seek urgent assessment.

4 Hands / feet

Capecitabine

Pain or peeling affecting daily activity: stop tablets and call.

6 Kidneys / urine protein

Bevacizumab

Urine protein is checked; swelling or reduced urine needs advice.

8 Bleeding / wounds

Bevacizumab

Report significant bleeding, clots and poor wound healing.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Stop capecitabine and contact oncology for significant diarrhoea, painful mouth or hand-foot symptoms affecting activity, vomiting that prevents drinking, fever or sudden illness. Do not restart after a toxicity interruption until the team advises. Severe abdominal pain, significant bleeding, sudden weakness or a severe unusual headache also need urgent assessment.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

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Keep your treatment details to hand

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Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Bevacizumab + capecitabine

Bevacizumab infusion on Day 1; capecitabine twice daily on Days 1–14, then no capecitabine on Days 15–21. Follow your individual prescription.

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Rolling record — follow your agreed treatment plan.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.